

Allergic Reaction/Anaphylaxis

History - Is the cause of allergy known? (Allergen) - Onset of symptoms after exposure? - History of prior reactions? - How severe? - Does patient carry an epinephrine autoinjector? - Did patient use it? - Other medical problems, especially cardiac or pulmonary?	Anaphylaxis - Moderate to Severe Allergic Reaction - Cutaneous or mucosal signs that occur suddenly, progress within minutes to hours, and are accompanied by respiratory compromise, hypotension, or persistent gastrointestinal symptoms. OR - Acute onset of hypotension or respiratory compromise, including severe bronchospasm or laryngeal involvement, even in the absence of skin involvement.
	Mild Signs and Symptoms - Generalized hives / itching with No other signs / symptoms - Normal Blood Pressure - Normal O2 saturation
EMT/AEMT - Follow General Medical Care Guideline Epinephrine Epinephrine Autoinjector 0.3 mg IM assist with own device <u>OR</u> Epinephrine 1:1,000, 0.5 mg IM lateral thigh (if not accessible then deltoid muscle) Albuterol 2.5 mg unit dose Nebulizer Treatment repeat as needed for wheezing - Follow Adult Resuscitation Guideline, as indicated - All patients treated with epinephrine should be evacuated and transported to a hospital - rebound reactions can occur up to 4-6 hours after initial epinephrine administration	
EMT/AEMT - Expanded Scope (see below in Paramedic section for indications) Diphenhydramine (Benadryl) PO or IM <u>OR</u> Loratadine PO - If giving Diphenhydramine, the patient should not return to the fireline for 4-6 hours or until no longer drowsy Steroid Prednisone 60mg PO <u>OR</u> Dexamethasone 10mg (IV solution as IM or PO) <u>OR</u> Solumedrol 125mg IM Famotidine 20mg PO	
Paramedic Mild Diphenhydramine (Benadryl) 25-50 mg PO/IM/IV <u>OR</u> Loratadine 10mg PO - If giving Diphenhydramine, the patient should not return to the fireline for 4-6 hours or until no longer drowsy Steroid Dexamethasone 10mg IV/IM/PO <u>OR</u> Prednisone 60 mg PO <u>OR</u> Solumedrol 125mg IV/IM Moderate to Severe - Next page	

If no improvement or rebound of symptoms after 2-3 minutes, Epinephrine 1:1,000, 0.5 mg IM, dose 2
 If no improvement or rebound of symptoms after 2-3 minutes, Epinephrine 1:1,000, 0.5 mg IM, dose 3

Allergic Reaction/Anaphylaxis - Continued

Paramedic

Moderate to Severe

- **Epinephrine** 1:1,000, 0.5 mg IM, if not given previously or if need for repeat dose
- **Diphenhydramine (Benadryl)** 25-50 mg PO/IM/IV OR **Loratadine** 10mg PO
 - If giving Diphenhydramine, the patient should not return to the fireline for 4-6 hours or until no longer drowsy
- **Steroid**
 - **Dexamethasone (Decadron)** 10 mg IV/IO/IM/PO OR
 - **Solumedrol 125mg** IV/IM OR
 - **Prednisone** 60mg PO
- **Famotidine** 20mg PO
- Albuterol 2.5 mg unit dose Nebulizer with Ipratropium Bromide 500 mcg, may repeat x 2
 - Additional Albuterol 2.5 mg / 3 mL continuous until symptoms resolve
- If no improvement or rebound of symptoms after 2-3 minutes, Epinephrine 1:1,000, 0.5 mg IM, dose 2
- If no improvement or rebound of symptoms after 2-3 minutes, Epinephrine 1:1,000, 0.5 mg IM, dose 3
- If refractory shock, Epinephrine Push Dose (1:100,000) 10-20 mcg (1-2 mL) IO/IV every 2-3 minutes PRN (max total 100 mcg / 10 mL)
 - Use only if patient remains in anaphylactic shock or airway compromise after IM epinephrine x 3 doses